

REGULATORY & COMPLIANCE

HIPAA Privacy Notice & Release of Information Pack

A model Notice of Privacy Practices, valid authorization forms, a records-request response kit, and a plain “who is asking?” guide.

AUDIENCE	FORMAT	VERSION	TIME TO ADAPT
Psychologists · Psychiatrists	Notice + forms + guide	1.0 · Sept 2026	1 to 2 hours

WHAT IT IS

The three privacy documents every practice needs: a Notice of Privacy Practices patients can understand, forms that let patients authorize disclosures properly, and a set of letters for responding to records requests, plus a guide to the common “someone is asking for information” situations.

USE IT WHEN

You are opening a practice, updating your notice, or have been asked for records by a patient, a family member, another clinician, an insurer or a lawyer.

CUSTOMIZE FIRST

Your practice name and contacts, privacy officer, state-law rules on minors and mental health records, and whether you handle substance use disorder records covered by 42 CFR Part 2.

WHAT CHANGED RECENTLY, AND WHAT HAS NOT

Practices that create or receive substance use disorder treatment records covered by 42 CFR Part 2 had to meet the updated rule and revise their notices by February 16, 2026. The 2024 reproductive-health privacy rule was vacated by a federal court in 2025, but the Part 2 changes to the notice were left in place, as we read HHS guidance. A proposed Privacy Rule update on coordinated care and access was still not final in the sources we reviewed in August 2026. Re-check [hhs.gov/hipaa](https://www.hhs.gov/hipaa) before you publish.

Part A: Notice of Privacy Practices (model)

A HIPAA-covered practice must give patients this notice at the first service, make a good-faith effort to obtain a signed acknowledgment, post it in the office, and put it on the website if you have one. Keep it in plain language. The text below covers the required content and is shorter than many templates. Adapt it and have it reviewed against your state law.

[Practice name] Notice of Privacy Practices. Effective **[date]**. This notice describes how health information about you may be used and disclosed and how you can get access to it. Please review it carefully.

Our duties. We are required by law to keep your health information private, to give you this notice, to follow it, and to tell you if there is a breach of your unsecured information.

How we may use and share your information without your written permission. (1) *Treatment*: with other providers involved in your care, for example a prescriber, when the law allows. (2) *Payment*: to bill you or your insurer. (3) *Health care operations*: for quality review, training and running the practice. We may also share information when required by law, for example to report suspected child or elder abuse, in response to a court order, to prevent a serious and imminent threat to someone's safety, for public health, health oversight, and as authorized for workers' compensation, law enforcement and coroners. State law may give you more protection than federal law, and we follow the stricter rule.

Uses that need your written authorization. Most uses of psychotherapy notes, marketing, and any sale of your information. Other uses not described here also need your written authorization. You can revoke an authorization in writing at any time, except for what we have already done in reliance on it.

Substance use disorder records. [If applicable] Records of substance use disorder treatment covered by 42 CFR Part 2 have additional protections. We may use and disclose them only as Part 2 permits, and they cannot be used against you in a legal proceeding without your written consent or a court order that meets Part 2's requirements. You may consent to future uses for treatment, payment and operations in a single consent, and you may revoke it.

Your rights. You have the right to: see and get a copy of your records (paper or electronic) within [30] days, subject to limited exceptions; ask us to correct them; ask for an accounting of certain disclosures; ask us to limit certain uses and disclosures (and we must agree if you pay in full out of pocket and ask us not to tell your health plan); ask us to contact you in a particular way or place; get a paper copy of this notice; and be told of a breach.

Complaints. If you think your privacy rights were violated, you can tell us at [privacy officer, phone, email] or complain to the U.S. Department of Health and Human Services, Office for Civil Rights, at hhs.gov/ocr/complaints. We will not retaliate against you for filing a complaint.

Changes. We may change this notice and the change will apply to information we already hold. The current notice will be posted in our office and at [website], and you may ask for a copy.

Part B: Acknowledgment of receipt

I received a copy of the Notice of Privacy Practices of [Practice name] on the date below.

Patient or representative signature

Relationship (if
representative)

Date

IF YOU CANNOT GET A SIGNATURE

HIPAA asks for a good-faith effort. If the patient declines or is unable to sign, note the date, what you tried and why you could not get it. Do not withhold treatment because a patient did not sign the acknowledgment.

Part C: Authorization to disclose or obtain information

A valid HIPAA authorization must be in plain language and contain specific elements. The form below includes them. Use a separate authorization for each recipient or purpose that differs, and give the patient a copy.

Patient name

Date of birth

Phone or email

Direction	Details
I authorize [Practice or clinician name] to release my information to:	Name and organization: _____ Address or fax: _____
OR I authorize the following to release my information to [Practice or clinician name]:	Name and organization: _____ Address or fax: _____

Information to be shared (check all that apply and be specific):

- ☐ Diagnosis and treatment plan
- ☐ Summary of progress or discharge summary
- ☐ Medication information
- ☐ Psychological or psychiatric evaluation or testing report
- ☐ Attendance and appointment information
- ☐ Full record, except psychotherapy notes
- ☐ Other: _____

Purpose (for example, coordination of care; at my request)

This authorization expires on (date or event)

If left blank, it ends one year from the date signed

My rights. I understand that: (1) I can revoke this authorization at any time by writing to [contact], except for actions already taken in reliance on it. (2) My treatment, payment, enrollment or eligibility for benefits does not depend on my signing it, except in the limited situations the law allows. (3) Information disclosed under this authorization may be redisclosed by the recipient and may no longer be protected by federal privacy law. (4) I am entitled to a copy of this form. [If Part 2 records are involved: describe the Part 2 statement required for consent.]

Patient (or personal representative) signature

Relationship and authority

Date

Authorization for psychotherapy notes (separate form)

Psychotherapy notes, as HIPAA defines them, need their own authorization that generally cannot be combined with any other. Keep this form separate and specific.

I authorize [clinician] to disclose my psychotherapy notes (as defined by HIPAA) to [recipient] for the purpose of [purpose] on [date range or expiration]. I understand I can revoke this in writing, that my treatment does not depend on signing, and that the recipient may redisclose the information.

Patient signature

Date

Part D: Responding to a patient's request for their records

Patients generally have a right to see and receive a copy of their records, in the form and format they ask for if you can readily produce it, including electronically. Federal rules give you 30 days to respond, with one 30-day extension if you send written notice of the reason and the new date. Your state may set a shorter time. Check the current rule, as proposals to shorten the federal period are pending.

Point	What to know
Ask why?	You cannot require the patient to explain why they want their records, or require a specific form or in-person pick-up.
Fees	You may charge a reasonable, cost-based fee for copying and postage, and for a summary if the patient agrees to one in advance. Check state limits.
Denial	Only on limited grounds. Psychotherapy notes and information compiled for legal proceedings are excluded from the right of access. Some denials, for example where access is reasonably likely to endanger life or safety, are reviewable and must be offered a review by a licensed professional who was not involved in the decision. Give a written denial that explains the basis and how to complain.
Summary instead of records	You may offer a treatment summary only if the patient agrees in advance. You cannot force it.
Third-party request from the patient	If the patient directs you in writing to send their electronic records to another person or app, you must generally do so.

Letter 1: Records provided

Dear [Patient first name], Enclosed are the records you requested on [date]: [description]. The fee for copying and postage is [\$___] (or: there is no charge). Please tell me if anything is missing. Some material may be excluded from access as the law allows. [If so, explain.] Sincerely, [Name]

Letter 2: Extension

Dear [Patient first name], I received your records request on [date]. I need more time to complete it. I will provide the records by [date, no more than 30 days after the original deadline]. The reason is [reason]. Sincerely, [Name]

Letter 3: Denial (in part or whole)

Dear [Patient first name], I am not able to provide [describe material] because [basis under law]. [If reviewable: You have the right to ask for review by a licensed professional who was not involved in this decision. To request it, contact ____.] You may also complain to me at [contact] or to the U.S. Department of Health and Human Services, Office for Civil Rights, at hhs.gov/ocr/complaints. The rest of the records you asked for are enclosed. Sincerely, [Name]

Part E: Who is asking? A quick guide

When in doubt, do not disclose. Ask what authority the requester is relying on, check your state law, get a written authorization or court order, and call your carrier's risk line.

Requester	General approach (state law may differ)
Another clinician treating the patient	HIPAA permits disclosure for treatment without authorization, but a signed release is best practice, and Part 2 and state law may require more (PA-08).
Spouse, partner or family member	Not without the patient's permission or another legal basis. You may listen. HIPAA lets you share limited information with family involved in care if the patient does not object, and in emergencies. Use judgment and document.
Parent or guardian of a minor	Depends on state law on the minor's consent to mental health care and on who is the personal representative. Decide your approach before treatment and put it in the consent (PA-09).
Insurer	For payment, limited to the minimum necessary. For audits and utilization review, follow the payer contract and give only what is requested.
Employer, school or agency	Only with the patient's written authorization, and only what the authorization describes.
Attorney or subpoena	A subpoena is not automatically a right to records. Follow the steps in PA-20.
Police or investigator	Ask for the legal basis and get it in writing. Follow PA-20 and your state law. In an emergency involving a serious threat, see PA-13.
Licensing board	HIPAA permits disclosure for health oversight. Respond within the deadline and contact your carrier immediately (PA-20).

IN PSYALLIANCE

Keep your own current Notice of Privacy Practices and authorization forms in your personal library. Share your practice-management questions and state-specific rules on minors' consent in the Town Hall for your state.

Sources and further reading

- HHS, HIPAA Privacy Rule: Notice of Privacy Practices, 45 CFR 164.520; authorizations, 164.508; individual access, 164.524 ([hhs.gov/hipaa](https://www.hhs.gov/hipaa)).
- HHS, Part 2 final rule fact sheet (compliance date February 16, 2026) ([hhs.gov/hipaa/for-professionals/regulatory-initiatives](https://www.hhs.gov/hipaa/for-professionals/regulatory-initiatives)).
- HHS OCR, Individuals' right under HIPAA to access their health information ([hhs.gov/hipaa/for-professionals/privacy/guidance/access](https://www.hhs.gov/hipaa/for-professionals/privacy/guidance/access)).
- HHS OCR, Modifications to the HIPAA Privacy Rule to Support Coordinated Care and Individual Engagement, proposed 2021, RIN 0945-AA00. Check current status.
- APA Ethics Code, Standards 4.01 to 4.07 (privacy and confidentiality) ([apa.org/ethics/code](https://www.apa.org/ethics/code)).

ABOUT THIS DOCUMENT

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